

SECTION C
DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK



Department of Defense
Defense Health Agency
Performance Work Statement

TRICARE Dental Program Six Generation (TDP6)

J-10/Deputy Assistant Director, Health Care Operations
Dental Program Section

Solicitation Number: HT9402-22-R-0001

Date: September 29, 2022

SECTION C DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

PART 1 1.0 GENERAL INFORMATION

C.1.1 This is a non-personal services contract to provide the TRICARE Dental Program sixth generation (TDP6).

C.1.2 Description of services/introduction: The TDP6 contractor shall provide comprehensive dental care services worldwide to eligible TRICARE beneficiaries. The contractor shall provide all personnel, equipment, supplies, facilities, transportation, tools, materials, supervision, and other items and non-personal services necessary to fulfill the requirements of TDP6 as defined in this Performance Work Statement (PWS) except for those items specified as government furnished property and services. The contractor shall perform to the standards in this PWS.

C.1.3 Background: The TRICARE Dental contract supports the delivery of the Department of Defense (DoD) dental benefit, which is codified in 10 U.S.C. Chapter 55 § 1076a. The TRICARE Dental contract serves to integrate affordable dental health services to DoD Military Health System (MHS) beneficiaries, by private sector contractors, and driven by greater integration of dental business processes across the MHS. The Defense Health Agency (DHA) supports the MHS in its mission to provide dental health services through TRICARE dental care coverage for all eligible family members of Uniformed Service active duty, members of the Selected Reserve and Individual Ready Reserve (IRR) and their eligible family members.

C.1.4 Objectives:

- The contractor shall provide worldwide cost effective dental care services for enrollees.
- The contractor shall promote the value of utilizing diagnostic and preventive services.
- The contractor shall provide data systems in which to monitor dental health plan data and outcome measures for all dental care rendered.

C.1.5 Scope: The TDP offers worldwide dental coverage to all eligible family members of Uniformed Service active duty personnel, and to members of the Selected Reserve, the Individual Ready Reserve (IRR), the National Guard, and their eligible family members. Active duty service members, former spouses, parents, in-laws, disabled veterans, foreign personnel, and retirees and their families are not eligible for the TDP. The TDP geographic area of coverage for the Continental United States (CONUS) includes the 50 United States, the District of Columbia, Puerto Rico, Guam and the U.S. Virgin Islands. Outside the Continental United States (OCONUS) service area includes all other countries, island masses and territorial waters.

In order to fulfill DHA's mission, TDP6 requires the contractor to establish and maintain a provider network in the 50 United States, District of Columbia, U.S. Virgin Islands, Guam, and Puerto Rico. In addition the contractor shall develop and maintain a list of Outside of the Continental United States (OCONUS) providers, referred to as TRICARE OCONUS Preferred Dentists (TOPDs).

The contractor shall be responsible for program operations and administration. TDP6 includes all associated administrative services, such as eligibility verification and enrollment, premium

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

collection, customer service, claims processing, and beneficiary and provider education, which are at a minimum, consistent with the contractor's commercial practices and industry standards.

C.1.6 Period of Performance (PoP):

Base Period/Transition-In: 12/01/2023 - 11/30/2024

Ordering Periods (OP) 1 through 5 will be:

OP 1: 12/01/2024 - 11/30/2025

OP 2: 12/01/2025 - 11/30/2026

OP 3: 12/01/2026 - 11/30/2027

OP 4: 12/01/2027 - 11/30/2028

OP 5: 12/01/2028 - 11/30/2029

The Residual Phase-Out CLIN (700X), will follow the last dental care delivery/transition out PoP.

Residual Phase-Out will begin after the last dental care delivery PoP (OP1, OP2, OP3, OP4, or OP5). The Government will exercise only one Residual Phase-Out CLIN (700X), which will depend on the last OP. The Residual Phase-Out CLIN's period of performance will have a one year PoP. The Residual Phase-Out PoP is for the contractor to complete processing of all claims, grievances, and appeals from the last OP of dental care delivery.

C.1.6.1 Transition-in/transition-out periods.

C.1.6.1.1 Transition-in period: Full performance start date is, 1 December 2024 Transition-in performance is defined at C.5.4.1 and Section J, Attachment J-11, Transition-In.

C.1.6.1.1.1 The contractor shall comply with transition-in requirements of the DHA, as listed in paragraph C.1.11.1, for contractors needing to be issued Common Access Card (CAC) identification, including Department of Defense (DoD) and DHA-directed training and forms submission, prior to network access.

C.1.6.1.2 Transition-out period: Transition-out performance is defined at C.5.4.2 and Section J, Attachment J-12, Transition-Out and Residual Services.

C.1.6.1.2.1 The contractor shall comply with transition-out requirements of the DHA for contractors who have been issued a CAC or who generate "records", as defined by C.5.5. Records Management.

C.1.7 Administrative Specifications

C.1.7.1 Place of performance: The contractor shall perform work at at the contractor's facility or in a remote environment as long as the required security precautions are documented and followed.

C.1.7.2 Recognized Federal holidays: The contractor is not required to perform services on the following holidays:

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

New Year's Day	Labor Day
Martin Luther King Jr.'s Birthday	Columbus Day
President's Day	Veteran's Day
Memorial Day	Thanksgiving Day
Juneteenth Day	Christmas Day
Independence Day	

C.1.7.3 Hours of operation: The contractor shall conduct business Monday thru Friday, with Federal holidays at the contractor's discretion or when the Government facility is closed due to local or national emergencies, administrative closings, or similar Government directed facility closings. The contractor's customer service center shall be available in accordance with Section J, Attachment J-5, Program Operations. The contractor shall at all times maintain an adequate workforce for the uninterrupted performance of all tasks defined within this PWS when the Government facility is not closed for the above reasons.

C.1.8 Contractor travel: The contractor shall be required to travel to meetings hosted by DHA in CONUS and OCONUS regions. The Government will provide a 14-day notice for all meetings hosted by DHA at one of its Government locations. The Government will host up to a maximum of four meetings as determined by the Government, per OP at its CONUS Government locations. In addition, the Government will host up to a maximum of two meetings as determined by the Government, per OP at each of its OCONUS regions: TRICARE Eurasia/Africa, TRICARE Latin America/Canada, and TRICARE Pacific. The Government will provide at least a 2-month notice for all OCONUS meetings that require travel. These meetings are in addition to those stated in Section J, Attachment J-11, Transition-In; and Attachment J-12, Transition-Out and Residual Services. All contractor travel costs associated with any of these meetings shall be the responsibility of the contractor.

C.1.10 Quality

C.1.10.1 Quality Control (QC): The contractor shall develop and maintain an effective QC program to ensure services are performed in accordance with this PWS. The contractor shall develop and implement procedures to identify, prevent, and ensure nonrecurrence of defective services. The contractor's QC program is the means by which the work complies with stated requirements. See C.5.6 Quality for additional information.

C.1.10.2 Quality assurance (QA): The government will evaluate the contractor's performance under this contract in accordance with the Quality Assurance Surveillance Plan (QASP). This plan provides a systematic method for the Government to evaluate performance and to ensure that the contractor has performed in accordance with the performance standards. It defines how the performance standards will be applied, the frequency of surveillance, and the minimum acceptable defect rate(s). See C.5.6.2 Contract Surveillance for additional information.

C.1.11 Contractor personnel

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

C.1.11.1 Contractor Common Access Card (CAC) requirements: For all contractors performing work on behalf of the Government, the Facilities Security Officer (FSO)/Company's Security point of contact (POC) shall provide the Government all the required information per the DHA CAC request process current version 2.1, January 2018, or more recent when updated. See Section J, Attachments J-13a, DHA CAC Request Process and J-16, TRICARE Service Manual (TSM), Chapter 1, Section 1.1. A CAC is the standard identification for eligible DoD contractor personnel.

C.1.11.2 Physical Security: The contractor shall be responsible for safeguarding all government information provided for contractor use (see C.5.2 System Security and TRICARE Operations Manual (TOM) Chapter 1, Section 5).

C.1.12 Contract Management and Administration: The contractor shall establish clear organizational lines of authority and responsibility to ensure adequate, effective resources are assigned to the contract requirements. The contractor shall have primary and alternate points of contact who are authorized to negotiate with the Government and commit to contract implementation and compliance (see Section G.3, Contract Administration). The contractor shall notify the Government within 5 business days of personnel changes to the contractor's leadership (e.g. president, vice president, etc.) and/or any changes to the primary and alternate points of contact.

C.1.12.1 The contractor shall establish and maintain effective management strategies, staff education and training programs, and coordination interfaces with the Government. The contractor shall comply with the directives cited in Part 2, paragraph C.2.3, Applicable Directives.

C.1.12.2 Out of Office and Closures: The contractor shall notify the the Contracting Officer's Representative (COR) 24 hours in advance (unless an emergency) when its primary and alternate points of contact will be out of the office for a full day or more.

C.1.13 Data rights: The contractor shall identify its proprietary data in accordance with the TOM, Chapter 9 Records Management and Section J, Attachment J-12, Transition-Out and Residual Services.

C.1.14 Reporting

C.1.14.1 Government's COR: The COR monitors all technical aspects of the contract and assists in contract administration. The COR is authorized to perform the following functions: assure that the contractor performs the technical requirements of the contract; perform inspections necessary in connection with contract performance; maintain written and oral communications with the contractor concerning technical aspects of the contract; issue written interpretations of technical requirements, including Government drawings, designs, specifications; monitor contractor's performance and notifies both the CO and contractor of any deficiencies; coordinate availability of government furnished property; and provide site entry of contractor personnel. A letter of designation issued to the COR, a copy of which is sent to the contractor, states the responsibilities and limitations of the COR, especially with regard to

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

changes in cost or price, estimates or changes in delivery dates. The COR is not authorized to change any of the terms and conditions of the resulting contract.

C.1.14.2 Post award conference/periodic progress meetings: The contractor shall attend any post award conference convened by the contracting activity or contract administration office in accordance with FAR Subpart 42.5. The Contracting Officer (CO), COR, and other Government personnel, as appropriate, may meet periodically with the contractor to review the contractor's performance (typically conducted via telephone). At these meetings the CO will apprise the contractor of how the government views the contractor's performance and the contractor will apprise the Government of problems, if any, being experienced. Appropriate action shall be taken to resolve outstanding issues. These meetings shall be at no additional cost to the government.

C.1.15 Contractor Identification

C.1.15.1 Contractors, including subcontractors at all tiers, shall provide for a clear distinction from Government personnel. Contractor employees shall not act, advertise, or presume to be Government employees, agents, or representatives. Contractor employees are required to appropriately identify themselves as contractor employees at all times, including in telephone conversations, formal and informal written correspondence, paper and electronic, and in any other situation where their actions could be construed as acts of Government officials, unless, in the judgment of the Government, no harm can come from failing to identify themselves. Contractor employees shall be introduced as contractor personnel and display distinguishing visible identification at all times whether in conversations, meetings, or other forms of communication with Government personnel.

C.1.15.2 Contractor personnel, while performing in a contractor capacity, shall refrain from using their retired or reserve component military rank or title (if applicable) in written or verbal communications associated with the contracts for which they provide services.

C.1.15.3 The contractor shall incorporate the substance of this requirement in all subcontracts awarded under this contract.

C.1.16 Contractor Access to Health Affairs (HA)/DHA Network(s)

C.1.16.1 FSO/Company's Security POC shall notify the DHA Personnel Security Office after being awarded a contract that requires access to a DoD system in accordance with Section J, Attachment J-16 TSM, Chapter 1, Section 1.1. Contractor personnel requiring access to the HA/DHA networks for performance of their tasks require a background investigation and the security awareness training. The contractor shall be prepared for this process as it could take two (2) or more weeks. The FSO/Security POC shall submit a Standard Form (SF) 85 to DHA's Personnel Security Office for a background investigation.

C.1.16.2 Company's FSO/Security POC must notify the Personnel Security Office when the contractor has submitted the SF-85. The FSO/Security POC must notify the DHA Personnel

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

Security Office in writing of a contractor's termination from the contract, including the termination date (see Section J, Attachment J-16 TSM, Chapter 1, Section 1.1).

C.1.17 Personnel Security

C.1.17.1 The contractor shall comply with DoDI 8582.01, “Security of Non-DoD Information Systems Processing Unclassified Non-Public DoD Information”, dated December 9, 2019; DoD Manual (DoDM) 6025.18, “Implementation of the HIPAA Privacy Rule in DoD Health Care Programs” dated March 13, 2019, DoDI 8580.02 Security of Individually Identifiable Health Information in DoD Health Care Programs, August 12, 2015; and DoDI 5200.02 “DoD Personnel Security Program (PSP),” March 21, 2014 (see Section J, Attachment J-16 TSM, Chapter 1, Section 1.1). Contractor responsibilities for ensuring personnel security include, but are not limited to, meeting the following requirements:

C.1.17.1.1 Follow the DHA Personnel Security Office guidelines for submittal of security clearances. Contact the DHA Personnel Security Office for guidance on the appropriate background investigation required for personnel on the contract. The DHA Personnel Security Office can be reached at (703) 275-6038.

C.1.17.1.2 Initiate, maintain, and document personnel security investigations appropriate to the individual’s responsibilities and required access to Controlled Unclassified Information (CUI) (see C.5.2 System Security).

C.1.17.1.3 DHA Personnel Security Office does not deny any access to any automated information system (AIS), network, or Controlled Unclassified Information (CUI). If a contractor receives an unfavorable background investigation, the request for access will be sent back to the FSO for further action. Any unfavorable adjudication will result in DHA Personnel Security Office not signing off on any access request.

SECTION C
DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

PART 2
2.0 DEFINITIONS, ACRONYMS, AND APPLICABLE
PUBLICATIONS/INSTRUCTIONS

C.2.1 Definitions: See Section J, Attachment J-1, Definitions and Acronyms, 32 CFR 199.13(b) and the TRICARE Operations Manual, Appendix A (<http://manuals.health.mil>).

C.2.2 Acronyms: See Section J, Attachment J-1, Definitions and Acronyms, and the TRICARE Operations Manual, Acronyms (<http://manuals.health.mil>).

C.2.3 Applicable Directives: The following documents form an integral part of this contract and have the same force and effect as if set forth in full text. The TRICARE Manuals provide instruction, guidance and responsibilities to establish contractual requirements for implementation of federal statutes and regulations. In the event of a conflict between the manuals and either statute or regulation, which are incorporated into this contract, the statute or regulation takes precedence. If there is a conflict between the manuals, the order of precedence is (1) TRICARE Policy Manual (TPM); (2) TRICARE Systems Manual (TSM); and (3) TRICARE Operations Manual (TOM). The TRICARE Manuals are located at <http://manuals.health.mil>. If the contractor believes that a conflict exists between the incorporated documents, the contractor shall inform the /CO.

C.2.3.1 Title 10, United States Code (U.S.C.), Chapter 55, Section 1076a.

C.2.3.2 32 Code of Federal Regulations (CFR) Part 199.3, 199.6, 199.9, 199.10 and 199.13.

C.2.3.3 36 CFR Part 1222 (data created or received and maintained for the Government by contractors).

C.2.3.4 45 CFR Parts 160, 162 and 164 (the Health Insurance Portability and Accountability Act of 1996 (HIPAA) security and privacy standards, transaction and code set standards, National Provider Identifier (NPI) requirements and implementation specifications).

C.2.3.5 National Institute of Standards and Technology (NIST) Special Publications (SP) 800-171.

C.2.3.6 TRICARE Policy Manual (TPM) 6010.60-M, April 1, 2015 through change 99. The following TPM Sections apply:

- Chapter 8, Section 13.1 Adjunctive Dental Care
- Chapter 8, Section 13.2 Dental Anesthesia
- Chapter 10, Section 5.1 paragraph 2.10 Dental Coverage
- Chapter 10, Section 9.1 – Early Eligibility Benefits for the Reserve Components (RCs)
- Chapter 10, Section 10.1 TRICARE Coverage for Certain Members of the National Guard (NG)

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

C.2.3.7 TRICARE Systems Manual (TSM) 7950.3-M, April 1, 2015, through change 61. The following TSM chapters apply:

- Chapter 1, General Automated Data Processing (ADP) Requirements (Section J, Attachment J-16, TSM Chapter 1, Section 1.1 will be utilized by the TDP contractor until it is published in the TSM.)
- Chapter 3, Defense Enrollment Eligibility Reporting System (DEERS) (Section 5.2 does not apply to TDP)

Note: The TRICARE Encounter Data (TED) requirement as stated in the TSM does not apply to TDP.

C.2.3.8 TRICARE Operations Manual (TOM) 6010.59-M, April 1, 2015, through change 110. The following TOM chapters apply:

- Chapter 1, Section 1 - Organization of the Defense Health Agency (DHA)
Section 2 - Contract Administration and Instructions (excluding paragraph 4.0 and 5.0)
Section 4 - Management (excluding paragraphs 2.2, 2.3, 2.4, 3.1, 3.2, 3.4 and 4.0)
Section 5 - Compliance with Federal Statutes
Section 6 - Legal Matters
- Chapter 2, Section 10 Other Requirements (only paragraph 6.0 Appeals and Hearings applies per Contract Data Requirements List (CDRL) R130 Appeals Processing Guidelines, Desk Instructions, and Reference Materials)
- Chapter 9, Records Management (Section 5, paragraph 3.0 Transfer to Other Contractors/Records Center, does not apply to TDP per Section J, Attachment J-12, Transition-Out and Residual Services, paragraph 3.18 Records Disposition)
- Chapter 11, Section 8 - Grievances and Grievance Processing
Addendum A TRICARE Logo
- Chapter 12, Appeals and Hearings
- Chapter 13, Program Integrity
- Chapter 14, Section 1 - Contract Data Requirements Lists (CDRLs), Plans and Access
- Chapter 19, Health Insurance Portability and Accountability Act (HIPAA) of 1996
- Chapter 24, Section 12 - TRICARE Overseas Program (TOP) Point of Contact (POC) Program
- Chapter 28 – Prescription Monitoring Program (PMP)
- Appendix A, Definitions
- Acronyms and Abbreviations may be located at <http://manuals.health.mil>

C.2.3.9 When a statute, regulation, or manual changes, the contract requirements shall not change until the Contracting Officer modifies the contract to incorporate the change. The contractor shall utilize the most current version of the TRICARE manuals for the requirements stated above (paragraphs C.2.3.6 through C.2.3.8). When a manual change is applicable to the contract, a contract modification will be issued to incorporate the change and update the manual references to reflect the latest manual change number. Manual change numbers may also be

SECTION C
DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

updated when other modifications are issued. In the event there are no applicable manual changes during an OP, an administrative modification may be issued annually to update the manual references to the most current manual change number.

SECTION C
DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

PART 3

3.0 GOVERNMENT FURNISHED PROPERTY, EQUIPMENT, AND SERVICES

The Requiring Activity Authority has assessed the need for Government Furnished Property, Equipment, and Services and determined:

C.3.1 Services: The Government will **NOT** provide Government Furnished Services in support of this contract/task order.

C.3.2 Facilities: The Government will **NOT** provide Facilities in support of this contract/task order.

C.3.3 Utilities: The Government will **NOT** provide Utilities in support of this contract/task order.

C.3.4 Equipment: The Government will **NOT** provide Equipment in support of this contract/task order.

C.3.5 Materials: The Government **IS** providing Materials in support of this contract/task orders. The Government-provided Materials are described below:

C.3.5.1 The Government will provide the contractor electronic data files containing relevant information pertaining to all sponsors. The Government will provide to the contractor a list of these data files and delivery times at the transition-in meeting with DMDC (see Section J, Attachments J-11, Transition-In). These data files will assist the contractor in determining their provider network adequacy and with beneficiary educational purposes. The contractor shall comply with the security requirements stated in C.5.2 prior to receiving the data files. Thereafter, a list of newly identified sponsors will be furnished Quarterly every OP.

C.3.5.2 The Government will provide the contractor the appropriate application software required to access the Defense Manpower Data Center's (DMDC) Defense Eligibility Enrollment Reporting System (DEERS) to perform eligibility inquiries and enrollments. The Government will grant the contractor access to DEERS only after the contractor's staff and all subcontractors' staff who will utilize systems which access and maintain TDP data, are compliant with the safeguarding methods for unclassified DoD information, personnel security and clearance requirements stated in C.5.1. The contractor and its subcontractors shall comply with C.5.1, no later than 120 days after award.

C.3.5.3 DMDC will notify all TDP enrollees that a new enrollment card is available on the milConnect.com site. Upon award of a new TDP contract, a new enrollment card will be generated for existing enrollees only if the TDP contract is awarded to a non-incumbent contractor.

C.3.5.4 The Government will print and provide to the contractor educational documents as identified in Section J, Attachment J-5, Program Operations.

SECTION C
DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

PART 4
4.0 CONTRACTOR FURNISHED ITEMS AND SERVICES

C.4.1 Services: The Contractor will **NOT** provide Contractor Furnished Services in support of this contract/task order.

C.4.2 General: The contractor shall furnish all supplies, equipment, facilities and services required to perform work listed under Part 5 of this PWS.

SECTION C
DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

PART 5
5.0 SPECIFIC TASKS

C.5.1 PERFORMANCE REQUIREMENTS

C.5.1.1 The contractor shall provide worldwide cost effective dental care services for enrollees.

C.5.1.1.1 The contractor shall comply with the most current version of American Dental Association's (ADA) Current Dental Terminology (CDT) manual for covered services and update its required systems.

Performance Standards

a) Standard: Whenever the ADA issues an updated CDT manual, the contractor shall identify CDT codes to be added, deleted and/or revised for Section J, Attachment J-2, Benefits, Exclusions and Limitations. The contractor shall provide the DHA Dental Program Section with its recommended changes for approval. The ADA updates CDT codes approximately once a year and the revisions are effective January 1 of the following year.

Acceptable Quality Level (AQL): The contractor shall submit its CDT recommendation within 30 calendar days of the release of the ADA's updated CDT manual.

b) Standard: The contractor shall update its systems to reflect the DHA Dental Program Section's approved CDT code changes no later than January 1 of the following year.

AQL: System updates shall be completed no later than January 1 100% of the time.

C.5.1.1.2 The contractor shall offer multiple avenues of access for continuous and open enrollment. Minimum avenues of access will be telephone, paper, and on-line enrollment. The members' enrollment must be able to transfer between active duty, Selected Reserve, IRR and National Guard status (see Section J, Attachments J-3, TDP Coverage Plans and J-5, Program Operations).

Performance Standards

a) Standard: Enrollment actions shall be processed and monitored in accordance with the TSM Chapter 3 and Section J, Attachment J-5, Program Operations.

Acceptable Quality Level (AQL): 99% of entries for telephone and paper enrollments will be accurate and complete.

b) Standard: If an enrollment application and premium payment are received after the 20th day of the month, enrollment will begin on the first day of the second month after the month in which the contractor received the application.

AQL: 95% of applications will be processed in accordance with the standard.

Deliverables

M010 Management Report

M070 Enrollment and Fee Reconciliation Analysis Report

Q030 Enrollment Activity Summary Report

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

S050 Survivor Covered Lives and Utilization Report

C.5.1.1.3 The contractor shall provide and maintain access to general and specialty network providers for enrollees.

Network compliance shall be determined based off the network provider's fixed structure location. "Driving miles" do not include alternative means of transportation (e.g. ferries, trains, or planes) in order for the beneficiary to access the provider's office. DHA defines:

- "Very Rural" is defined as less than 300 residents per square mile.
- "Rural" is defined as more than 300 to 1,000 residents per square mile.
- "Suburban" is defined as more than 1,000 to 3,000 residents per square mile.
- "Urban" is defined as more than 3,000 residents per square mile.

The contractor may utilize the Census Bureau for determining the number of residents of the above stated areas. See Section H.3.1.3, Network Provider Retention incentive, and Section J, Attachment J-5, Program Operations for provider requirements.

Performance Standards

a) Standard: The contractor shall establish networks for general dental care in the 50 United States, District of Columbia, U.S. Virgin Islands, Guam, and Puerto Rico.

AQL: (1) For Very Rural: 95% of enrollees shall have access to one general dentist within 21 calendar days of requesting a periodic or initial (non-emergency) appointment and within 35 driving miles of their place of residence.

(2) For Rural: 95% of enrollees shall have access to two general dentists within 21 calendar days of requesting a periodic or initial (non-emergency) appointment and within 25 driving miles of their place of residence.

(3) For Suburban: 96% of enrollees shall have access to three general dentists within 21 calendar days of requesting a periodic or initial (non-emergency) appointment and within 15 driving miles of their place of residence.

(4) For Urban: 96% of enrollees shall have access to four general dentist within 21 calendar days of requesting a periodic or initial (non-emergency) appointment and within 5 driving miles of their place of residence.

b) Standard: The contractor shall establish networks for specialty dental care in the 50 United States, District of Columbia, U.S. Virgin Islands, Guam, and Puerto Rico.

AQL: (1) 90% of enrollees shall have access to an orthodontist within 28 calendar days of requesting an appointment and within 35 driving miles of their place of residence.

(2) 95% of enrollees shall have access to an oral surgeon within 28 calendar days of requesting an appointment and within 35 driving miles of their place of residence.

(3) 85% of enrollees shall have access to an endodontist within 28 calendar days of requesting an appointment and within 35 driving miles of their place of residence.

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

(4) 85% of enrollees shall have access to a periodontist within 28 calendar days of requesting an appointment and within 35 driving miles of their place of residence.

(5) 60% of enrollees shall have access to a prosthodontist within 28 calendar days of requesting an appointment and within 35 driving miles of their place of residence.

c) Standard: The contractor shall maintain a network provider retention rate in accordance with Section J, Exhibit A, Contract Data Requirements List (CDRL) M020, Provider Network Access Report. For OP 1 the COR will validate the baseline 6 months after start of dental care delivery. The baseline shall be reset at the beginning of each OP, and the Contracting Officer Representative (COR) will re-validate at the end of the OP.

AQL: 95% of network providers shall be retained annually.

d) Standard: The contractor shall deploy Mobile Dental Units (MDUs) in accordance with Section J, Attachment J-5, Program Operations for areas in which there are no practicing dentists.*

AQL: MDUs shall be deployed once in each OP for up to five consecutive days to a centrally-located area within each identified county to serve those 200 or more enrollees whose closest practicing dentist is located 50 or more miles from the enrollee's place of residence or the enrollees reside in a non-compliant area.

*The standard applies until such time a practicing dentist (network or non-network) resides in the location and access standards are met. A minimum of 30% of TDP enrollees must be scheduled for mobile care in each location per event for a mobile unit to be deployed and the standard to apply.

Deliverables

M020 Provider Network Access Report

C.5.1.1.4 The contractor shall develop, maintain and make available to enrollees, providers and Government representatives an up-to-date directory of network providers.

Performance Standard

a) Standard: The contractor shall post the updated directory on the contractor's website in accordance with Section J, Attachment J-5, Program Operations.

AQL: 100% updated within 5 business days of receipt of the updated information.

Deliverables

M020 Provider Network Access Report

C.5.1.1.5 The contractor shall develop and maintain agreements with TRICARE OCONUS Preferred Dentists (TOPDs).

Performance Standards

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

a) Standard: The contractor shall complete agreements with providers identified by the Government and for any new providers identified each OP in accordance with Section J, Attachments J-6a, OCONUS Program Operations, and J-11, Transition-In.

AQL: The contractor shall complete agreements with 95% of providers identified by the DHA COR prior to OP 1 and any new providers identified each OP.

b) Standard: The contractor shall make available to enrollees, TOPDs and Government representatives an up-to-date list of TOPDs in accordance with Section J, Attachment J-6a, OCONUS Program Operations.

AQL: 100% updated within 5 business days of receipt of the updated information.

Deliverables

W010 Transition-In Status Report

M010 Management Report

C.5.1.1.6 The contractor shall process claims to completion for enrollees and providers.

Process to completion is defined in the TOM, Appendix A.

Performance Standards

a) Standard: Process CONUS claims to completion in accordance with Section J, Attachment J-5, Program Operations.

AQL: CONUS Claims

95% within 14 business days of receipt

98% within 30 business days of receipt

100% within 60 business days of receipt

b) Standard: Process OCONUS claims to completion in accordance with Section J, Attachment J-6a, OCONUS Program Operations.

AQL: OCONUS Claims

95% within 21 business days of receipt

98% within 30 business days of receipt

100% within 60 business days of receipt

c) Standard: Claims shall be accurately paid in accordance with the covered benefit. CONUS and OCONUS claims shall be separately tracked.

AQL: 97% are accurately paid

d) Standard: Claim errors shall be corrected. CONUS and OCONUS claims shall be separately tracked.

AQL: 99% are corrected

Deliverables

M010 Management Report

M030 OCONUS Payment Report

M050 Worldwide Incurred/Paid Claims Report

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

S010 Utilization by Uniformed Services Active Duty (AD) Report
S020 Utilization by Uniformed Service National Guard Reserve (NGR) Report
S030 Utilization CONUS/OCONUS Frequency & Dollar Report
S040 Utilization CONUS/OCONUS Denied Services Report
S050 Survivor Covered Lives and Utilization Report

C.5.1.1.7 The contractor shall process appeals to completion for enrollees and providers.

Performance Standards

a) Standard: Appeals shall be processed in accordance with 32 CFR 199.13(h) and the TOM, Chapter 12 (Use Section J, Attachment J-9, Appeals Summary Log in place of Figure 12.A-2 of the TOM).

b) Standard: The contractor shall process non-expedited medical necessity and factual reconsiderations to completion.

AQL: 90% within 30 calendar days of receipt
95% within 60 calendar days of receipt
100% within 90 calendar days of receipt

c) Standard: The contractor shall process to completion determinations reversed by appeals.

AQL: 100% within 21 calendar days of receipt

Deliverables

M010 Management Report
R120 Appeals Processing Guidelines, Desk Instructions and Reference Materials

C.5.1.1.8 The contractor shall process grievances to completion for enrollees and providers.

Performance Standards

a) Standard: The contractor shall process grievances to completion in accordance with TOM, Chapter 11, Section 8.

AQL: 95% within 60 calendar days of receipt

Deliverables

M010 Management Report

C.5.1.1.9 The contractor shall provide readily accessible comprehensive customer service for beneficiaries and providers.

Performance Standards

a) Standard: The contractor shall provide customer service in accordance with Section J, Attachment J-5, Program Operations.

AQL: (1) 98% of telephone calls answered by Automated Response Unit within 30 seconds.

SECTION C
DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

- (2) 80% of telephone calls answered by Customer Service Representative (CSR) within 30 seconds of selection by caller.
- (3) 5% or less telephone calls blocked at all times (measured at least hourly)
- (4) 80% of telephone calls resolved during the initial call; 99.9% within 7 business days.
- (5) Priority Written and E-mail Correspondence (final response). Priority correspondence is that which is received from Members of Congress, DoD leadership and DHA leadership.
95% within 7 business days of receipt
99.9% within 20 business days of receipt.
- (6) Non-priority (routine) Written and E-mail Correspondence (final response).
85% within 10 business days
99.9% within 30 business days.

b) Standard: The contractor shall maintain overall customer satisfaction with TDP. The measurement will be based on the annual average of the quarterly customer satisfaction phone surveys conducted by DHA, Defense Health Cost Assessment and Program Evaluation (DHCAPE). The results will be provided to the contractor within 30 calendar days of completing each of the quarterly surveys.

AQL: At least 95% of those surveyed will indicate that, overall, they are at a minimum “somewhat satisfied”.

Deliverables

M010 Management Report (applies only to Standard ‘a’ above)

C.5.1.1.10 The contractor shall develop and maintain a Quality Control Program.

Performance Standard

a) Standard: The contractor shall develop and implement procedures to identify, prevent, and correct problems in accordance with Section C.5.6 Quality.

AQL: 99% of sampled claims shall indicate appropriate standard of care was rendered each OP.

Deliverables

Q010 Quality Control Program (QCP) Report

C.5.1.2 The contractor shall promote the value of utilizing diagnostic and preventive services.

C.5.1.2.1 The contractor shall inform and educate beneficiaries and providers about the program benefits with emphasis to the beneficiaries about the benefit of utilizing diagnostic and preventive services.

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

Performance Standard

a) Standard: The contractor shall comply with its education plan in accordance with Section J, Attachments J-5, Program Operations and J-6a, OCONUS Program Operations. The contractor shall allow 30 calendar days for the DHA Dental Program Section to approve/deny any changes to the contractor's education plan. Timing of the publication and/or implementation shall be mutually agreed upon between the contractor and DHA Dental Program Section.

AQL: 100% of planned education activities shall be completed within the current OP.

Deliverables

AP010 Beneficiary and Provider Education Plan

R110 MOU with DHA Communications

C.5.1.3 The contractor shall provide data systems in which to monitor dental health plan data and outcome measures for all dental care rendered.

C.5.1.3.1 The contractor shall provide training for the users on the data systems stated in Section C.5.8, Data Systems Access and Training.

Performance Standards

a) Standard: The contractor shall train (initial and refresher) and provide secure means for the users to access the systems for viewing and/or utilization. The contractor shall provide a dedicated manager responsible for training, issues, process suggestions, and provide assistance to the user when requested. The contractor may provide the training on-site or electronically.

AQL: (1) Initial system training conducted during the Transition-In period shall be completed no later than 60 calendar days prior to OP 1.

(2) After start of OP 1, new users shall receive initial training no later than 15 calendar days after identification is made to the contractor.

(3) Refresher training shall be provided quarterly.

(4) Provide refresher system training webinar within seven days of a request.

(5) Reply to data system inquiries within one business day.

Deliverables

M010 Management Report

C.5.1.3.2. The contractor shall provide Government designated users with read-only access to the contractor's full TDP data set.

Performance Standard

a) Standard: The user shall have the ability to download any of the data elements for manipulation purposes in accordance with Section C.5.8.2, TDP Full Data Set.

AQL: The system shall be available 99.9% of the time 24 hours, 7 days a week (excluding downtime for scheduled maintenance).

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

Deliverables

M010 Management Report

C.5.1.3.3 The contractor shall provide Government designated users with read-only access to a TDP dashboard.

Performance Standard

a) Standard: The TDP data shall be provided in a consolidated manner in which the user shall have the ability to easily view and download dental health plan data and the outcome measurements without having to manipulate the data in order to retrieve the information in accordance with Section C.5.8.3, TDP Dashboard.

AQL: The system shall be available 99.9% of the time 24 hours, 7 days a week (excluding downtime for scheduled maintenance).

Deliverables

M010 Management Report

C.5.1.3.4 The contractor's system shall interface with the Military Health Services (MHS) Genesis/Defense Health Medical Systems Modernization (DHMSM) Electronic Health Record System (EHRS).

The contractor shall implement the MHS Genesis/DHMSM EHRS at a later date as directed by DHA. DHA will provide the MHS Genesis/DHMSM EHRS requirements will be incorporated into the contract prior to implementation.

C.5.2 System Security. The contractor shall acquire, develop, and maintain processes for safeguarding unclassified DoD information, including covered defense information, commonly referred to as DoD controlled unclassified information on all contractor/subcontractor systems/networks that store, process, transmit or access unclassified DoD information or controlled unclassified information in accordance with TSM, Chapter 1, Section 1.1 (see Section J, Attachment J-16 until publication of the revised TSM, Chapter 1, Section 1.1 and also Section C, Part 6). The contractor shall implement and maintain adequate security in its project, enterprise, or company-wide internal information systems or covered contractor information systems that process, store, create, or transmit unclassified DoD information in accordance with the basic safeguarding requirements in FAR 52.204-21. The contractor shall implement and maintain adequate security in its project, enterprise, or company-wide internal information systems or covered contractor information systems that process, store, create, or transmit covered defense information, commonly referred to as controlled unclassified information (e.g. PII/PHI), in accordance with safeguarding covered defense and cyber incident reporting requirements in DFARS Clause 252.204-7012. Covered contractor information systems shall be subject to the security requirements in National Institute of Standards and Technology (NIST) Special Publication (SP) 800-171, "Protecting Controlled Unclassified Information in Nonfederal Information Systems and Organizations", Privacy Act Program Requirements (DoD 5400.11-R), and the Personnel Security Program (DoD 5200.2-R). The contractor's annual System Security

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

Plan, or extract thereof, with any applicable Plans of Action is required prior to accessing DoD data, interfacing with Government systems, or conducting testing.

Deliverables

M080 Employee Access to DoD IS/Networks Report

A030 Disaster Recovery Test Results Report

AP020 Continuity of Operations Plan (COOP)

AP050 System Security Plan and Associated Plans of Action for Contractor's Internal Unclassified Information System

C.5.3 Health Insurance Portability and Accountability Act (HIPAA). The contractor shall comply with the HIPAA requirements of TOM, Chapter 19.

Deliverables

M040 HIPAA/Privacy Complaint Report

A020 Risk Assessment Letter of Assurance

R010 Breach Report

R070 Designated Standards Maintenance Organization (DSMO) Meeting Summary Report

C.5.4 Transition.

C.5.4.1. The transition-in of this contract shall be conducted in accordance with the requirements stated in Section J, Attachment J-11, Transition-In.

Deliverables

W010 Transition-In Status Report

R030 Transition-In Integrated Master Plan (IMP) and Integrated Master Schedule (IMS)

R170 B2B Gateway Questionnaire

C.5.4.2. The transition-out and the residual services activities shall be conducted in accordance with Section J, Attachment J-12, Transition-Out and Residual Services. Transition-Out costs shall not apply, nor shall any payment be made on CLINs X0010, if an incumbent contractor is selected as the awardee for the follow-on contract.

Deliverables

M090 Transition-Out and Residual Services Status Report

R040 Transition-Out and Residual Services Plan

C.5.5 Records Management. All contractor records generated under this contract shall be maintained in accordance with 36 CFR 1222; TOM, Chapter 9; Section J, Attachment J-11, Transition-In; and Attachment J-12, Transition-Out and Residual Services.

Deliverables

R130 Declaration of Transfer and Destruction of Records

C.5.6 Quality

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

C.5.6.1 Quality Control Program (QCP). The contractor shall develop and implement a QCP in accordance with the TOM, Chapter 1, Section 4 (excluding paragraphs 2.2, 2.3, 2.4, 3.1, 3.2, 3.4 and 4.0) and Section J, Exhibit A, CDRL AP030, QCP Plan. The contractor shall maintain a QCP that ensures that services are performed in accordance with the contract. The contractor shall develop and implement procedures to identify, prevent, and correct problems for the duration of the contract (see Section C.5.1.1.10). The COR will review the contractor's sampling methodology after award to determine the minimum sampling size for C.5.1.1.10.

Deliverables

Q010 Quality Control Program (QCP) Report
AP030 Quality Control Program (QCP) Plan

C.5.6.1.1 Clinical Quality. The contractor shall ensure that their QCP Plan includes written policies and procedures to identify and review all potential quality issues (PQIs) from any source, to include grievances. A quality issue (QI) is a verified deviation, as determined by a qualified reviewer, from acceptable standards of practice or care as a result of some process, individual, or institutional component of the health care/dental care system. All QIs, regardless of the source, shall be reviewed and confirmed by qualified staff to determine deviations from standards of care, assign severity levels, recommend interventions to include Corrective Action Plans, report to licensure boards, and follow-up monitoring through resolution. All standard of care determinations shall be approved by the contractor's head of clinical dental care. The contractor shall assure all identified issues are tracked and trended and any patterns are identified; all issues must be appropriately addressed until complete resolution is achieved. The contractor shall ensure that no results from a quality review (quality assurance information) are disclosed to any person or entity unless specifically provided for under 10 USC §1102.

Deliverables

M010 Management Report
Q010 Quality Control Program (QCP) Report
AP030 Quality Control Program (QCP) Plan

C.5.6.2 Contract Surveillance. The Contracting Officer (CO) and Contracting Officer's Representative (COR) will develop a Quality Assurance Surveillance Plan (QASP) to define acceptable quality levels and identify how the COR will monitor the quality of the contractor's performance and address contractor performance that falls below these standards. The CO and COR will finalize the QASP after award and provide a copy to the contractor (for purposes of the RFP see Section J, Attachment J-19, Draft QASP). The QASP is a living document, and the Government may unilaterally update it as necessary. The CO and COR will complete an annual contractor's performance evaluation in the Contractor Performance Assessment Report System (CPARS).

C.5.7 Fraud and Abuse

C.5.7.1 The contractor shall implement a Fraud and Abuse program in accordance with TOM, Chapter 13.

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

Deliverables

Q020 Fraud and Abuse Summary Report
A040 Fraud Detection and Prevention Strategy and Internal Procedures
R060 Threats Report
R090 Standard Operating Procedures (Desk Procedures)
R140 Random Sample Audit Worksheet
R150 Fraud/Abuse Patient Harm-Initial Notification Checklist
R160 DHA/MTF Fraud and Abuse Referral Cover Sheet

C.5.7.2 Potential Fraud and Abuse Cases. The contractor shall refer to DHA any potential case that involves more than a \$10,000 loss to the Government and any potential case involving patient harm regardless of loss value (see TOM, Chapter 13, Section 3 for further requirements).

C.5.7.3 Post Payment Utilization Reviews. When performing post payment utilization reviews, the contractor shall consider high volume beneficiaries as those beneficiaries whose charges exceed \$25,000 during a 12-month reporting period. High volume providers are considered institutional providers whose payments exceed \$750,000; individual providers whose payments exceed \$100,000; group/clinics whose payments exceed \$100,000 (the threshold average is \$25,000 per professional provider within the group) during a 12-month reporting period (see TOM, Chapter 13, Section 4 for further requirements).

C.5.8 Data Systems Access and Training

C.5.8.1 The contractor shall provide access to the full TDP data set and the TDP dashboard (stated below) for the DHA Dental Program Section, Contracting Officer, Contract Specialist, Contracting Officer's Representative, and the Tri-Service Center for Oral Health Studies (TSCOHS), for a total of 10-15 user accounts. The DHA Dental Program Section will provide the contractor a list of users during Transition-In. The contractor shall train (initial and refresher) and provide secure means for the POCs to access the system(s) for viewing and/or utilization in accordance with Section C.5.1.3.1. The contractor shall provide the Transition-In initial training at the contractor's place of business. After start of OP1, the contractor shall offer options in which to attend new user initial training and refresher training (e.g. the contractor's place of business, webinar, Teams, etc.). The contractor shall provide a dedicated manager responsible for training, issues, process suggestions, and provide assistance to the user when requested.

C.5.8.1.1 All contractor's data systems shall be available in accordance with Section C.5.1.3.2 and C.5.1.3.3., excluding scheduled maintenance. To the maximum extent practicable, the contractor shall schedule system maintenance windows during weekends or non-peak hours to minimize disruption of services to Government users. To the extent practicable, the contractor shall notify the DHA, Dental Program Section 24 hours in advance of system maintenance if it will disrupt services during peak hours

C.5.8.2 TDP Full Data Set. The contractor shall provide the Government users, as stated in C.5.8.1, with read-only access to their full TDP data set with download capability for data

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

manipulation purposes with the ability to download all data elements associated with the TDP contract. The data system shall allow the Government the ability to parse and manipulate the data by downloading only the specific data elements desired and run reports using only those specific data elements. Access shall be on-line, and the data and any updates shall be available as soon as the source system provides it.

C.5.8.3 TDP Dashboard. The contractor shall provide a dashboard that provides current performance information on the TDP contract and monitors outcome measures. The TDP dashboard shall be fully operational the first day of OP1. The TDP dashboard shall allow the Government the ability to trend and extrapolate specific information with visualization graphic display and analytic options. The TDP dashboard shall be provided in a consolidated manner in which the DHA Dental Program Section may easily view the data without having to manipulate the data in order to retrieve the information. The information shall be easy to access, for example: point and click on a graphic; enter a ZIP code or state/country to see network access rates; identify how many enrollees and providers are in a location; or view any of the performance standards. The data shall be as current as possible and shall be totaled monthly and summarized quarterly.

C.5.8.3.1 The TDP dashboard shall include, but not be limited to, information on network providers, non-network providers, TRICARE Overseas Preferred Dentists (TOPDs), enrollment information, claims processing, claims payment, enrollee dental care and service data. DHA may choose to add an item at any time during the contract and the contractor shall ensure access to that data within 60 days (unless otherwise agreed upon by DHA and the contractor) of the request at no additional cost to the contract. The TDP dashboard shall allow for monitoring and displaying outcome measures and trends for the TDP contract:

- monitor provider access and satisfaction
- utilization by CDT code and by user selected group
- effectiveness of care
- enrollment by plan type, military service, and age group
- utilization of prophylaxes of pregnant and diabetic enrollees
- prophylaxes utilization by individual enrollee
- utilization of scaling and root planing of diabetic enrollees
- utilization of the dental accident coverage, and
- number of enrollees reaching the annual maximum

C.5.9 Military Health System (MHS) Data Repository (MDR) and MHS Mart (M2).

C.5.9.1 MDR is DHA's centralized data repository that captures, archives, validates, integrates and distributes DHA corporate health care data worldwide. It receives and validates data from the DoD's worldwide network of more than 260 healthcare facilities and from non-DoD data sources. M2 is DHA's management and reporting tool. Each DHA contractor and DHA must program their systems to transmit and receive claims payment data to MDR and M2. The contractor shall provide dental care encounter data for input in the MDR. The data, which includes both clinical and financial information, will then be available for reporting and inquiry purposes in the MDR. The data will also be available on the reports residing on the M2.

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

C.5.9.2. The contractor shall work with the DHA Dental Program Section and DHA, Defense Health Services Systems (DHSS) in developing a TDP Interface Control Document (ICD) describing the data exchange to the MDR. The purpose of the TDP ICD is to describe the interface that provides the TDP records from the contractor's automated information systems in support of the TDP (see Section J, Attachment J-11, Transition-In). The ICD shall be completed 45 days prior to OP1.

C.5.9.3 The contractor shall submit the claims and provider data in accordance with the ICD and Section J, Attachment J-10, MDR Data Elements Layout.

C.5.9.4 The contractor shall protect the data in accordance with the C2-level protection standards mandated for all "Sensitive Unclassified Systems" as required in the DoD Directive 5200.28 since the data exchanged in this interface contains protected patient level identifiable information and the aggregate data being transmitted by DHSS becomes part of a database that contains sensitive data.

C.5.9.5 The contractor shall perform validation checks such as record counts, file formats, source stamps, and date-time stamps on data transferred from the contractor to the MDR as defined in the MDR ICD. When errors are discovered in the data exchange, the contractor will be notified immediately by DHSS operations personnel. If there are systemic problems, Interface Working Group (IWG) counterparts will be contacted by DHSS to work the issues.

Deliverables

M060 MDR Data Claims and Provider Files

C.5.10 Deliverables.

C.5.10.1 The contractor shall provide all reports and plans in accordance with Section F.5, Reports and Plans.

C.5.10.2 Contract Data Requirements List (CDRLs). The contractor shall work with the Government during transition on the report formats for the CDRLs (see Section J, Exhibit A, CDRLs). The contractor shall work with the Government to fully describe the format/data elements (i.e., field descriptions, field lengths, list of allowed entries for each field) to be used for each report and to build/test the reports to be forwarded to the Government. During the term of the contract, the CO or the designee may require additional changes to the makeup of these reports (e.g., add or delete data elements or change the format). The contractor shall implement these changes at no additional cost to the Government.

C.5.10.3 Special Reports and Plans.

C.5.10.3.1 CLIN Price Structure Report. The contractor shall provide its price structure build-up and supporting documentation for its Section B CLIN unit prices after award.

Deliverables

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

R180 CLIN Price Structure

C.5.10.3.2 Ad Hoc Reports. The Government may request the contractor to submit up to eight ad hoc reports per OP at no additional cost to the Government. The contractor shall provide the report within 2 weeks of the request unless a revised date is mutually agreed upon with DHA.

Deliverables

R080 Ad Hoc Management Reports

C.5.10.3.3 Consolidated Historical Data File. The Government may request the contractor to submit a historical data file up to two times over the life of the contract.

Deliverables

R100 Consolidated TDP Historical Data File

C.5.10.3.4 Continuation of Essential Contractor Services. The contractor shall develop and maintain a plan describing how it will continue to perform the essential contractor services described in attachments J-5 Program Operations and J-6 OCONUS Program Operations, during periods of crisis as described in Section I, DFARS 252.237-7023 Continuation of Essential Contractor Services. The plan shall include procedures to identify, prevent, and correct problems for the duration of the contract.

Deliverables

AP040 Continuation of Essential Contractor Services Plan

C.5.10.3.5 Disaster Action Plan. The contractor shall develop a Disaster Action Plan to be implemented when the President of the United States declares an area of the United States or any U.S. territory with a provider network to be a "National Disaster Area" (see Section J, Attachment J-5, Program Operations).

Deliverables

R050 National Disaster Action Report

C.5.10.3.6. Proposed Freedom of Information Act (FOIA) Releasable Contract. The contractor shall provide a bracketed copy of the original signed, executed contract with attachments and exhibits for the purpose of DHA having a releasable redacted version for FOIA requests.

Deliverables

A010 Proposed Freedom of Information Act (FOIA) Releasable Contract

SECTION C
DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

PART 6
6.0 INFORMATION TECHNOLOGY & SECURITY

C.6.1 All work under this contract is unclassified.

C.6.2 The TIER 1, TIER 2, or TIER 3 levels and position sensitivity designation for positions under this contract is: TIER 3: Non-Critical sensitive position (see Section J, Attachment J-16, TSM Chapter 1 Section 1.1).

C.6.3 Personally Identifiable Information (PII)/Protected Health Information (PHI), Procurement, and Federal information requirements:

C.6.3.1. Data Sharing Agreements (DSAs): Contractors requiring access to PII, which includes PHI, or access to de-identified data, are subject to the DHA Privacy and Civil Liberties Office (DPCLO) (Privacy Office) Data Sharing Program and shall comply with Section J, Attachment J-16, TSM Chapter 1 Section 1.1.

C.6.3.2. Processing Procurement Sensitive Information: The contractor shall seek guidance from the CO regarding the coordination of documents, dissemination, and transmission of procurement sensitive information. Procurement sensitive information shall not be transmitted electronically unless encryption is utilized. Depending on a particular procurement, other restrictions may apply.

C.6.4 Training – See Section J, Attachment J-16, TSM Chapter 1 Section 1.1 for more information.

C.6.5 Cybersecurity Requirements for Non-DoD IT or Covered Contractor IS: The contractor shall, at time of award, have implemented the security requirements prescribed in the National Institute of Standards and Technology (NIST) Special Publication (SP) 800-171, “Protecting Controlled Unclassified Information in Nonfederal Information Systems and Organizations” (available via the internet at <http://dx.doi.org/10.6028/NIST.SP.800-171>), in accordance with DFARS clause 252.204-7012. See Section J, Attachment J-16, TSM Chapter 1 Section 1.1 for additional information.

C.6.5.2 Cyber Incident Reporting Requirement: When the contractor discovers a cyber incident that affects a covered contractor information system or the covered defense information residing therein, or that affects the contractor’s ability to perform the requirements of the contract that are designated as operationally critical support and identified in the contract, the contractor shall adhere to the incident reporting and handling procedures prescribed in DFARS clause 252.204-7012.

C.6.5.3 The contractor shall maintain within the US or US territories all Government data that is not physically located on DoD premises, unless the contractor receives written notification from the CO to use another location, in accordance with DFARS 239.7602-2(a).

SECTION C

DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

C.6.5.4 The contractor shall mitigate supply chain risk to the government by complying with DFARS 252.239-7018 and only utilizing unified capability equipment identified on the DODIN Unified Capabilities Approved Products List (<https://aplits.disa.mil/processAPList>), unless granted a waiver in accordance with DODI 8100.04, DOD Unified Capabilities (UC).

C.6.6 Telecommunications:

C.6.6.1 MHS Demilitarized Zone (DMZ) Medical Community of Interest (MedCOI) Business-to-Business (B2B) Gateway. If there is an external system connection to a DoD IS required, then the contractor shall, in accordance with contract requirements, connect to the DHA B2B Gateway via a contractor procured Internet Service Provider (ISP) connection (see Section J, Attachment J-16, TSM Chapter 1 Section 1.1 for additional information).

C.6.6.2 Contractor Provided IT Infrastructure. The contractor shall ensure its platforms support interconnectivity in accordance with Section J, Attachment J-16, TSM Chapter 1 Section 1.1

C.6.6.3 System Authorization Access Request (SAAR), Defense Department (DD) Form 2875. If a contractor uses DoD Gateways to access Government IT systems and/or DoD applications, the contractor shall submit the most current version of DD Form 2875, "System Authorization Access Request (SAAR), in accordance with Section J, Attachment J-16, TSM Chapter 1 Section 1.1.

C.6.6.4 MHS Systems Telecommunications. The contractor shall ensure primary communication links are via encrypted tunnels (i.e., Secure Internet Protocol (IPSEC), GetVPN, or Secure Socket Layer) between the contractor's primary site and the MHS B2B Gateway in accordance with Section J, Attachment J-16, TSM Chapter 1 Section 1.1.

C.6.6.5 Establishment of Telecommunications. The contractor shall establish telecommunications with the MHS in accordance with Section J, Attachment J-16, TSM Chapter 1 Section 1.1.

SECTION C
DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

PART 7
7.0 ATTACHMENTS/TECHNICAL EXHIBIT LISTING

For a complete list of all attachments and exhibits, see Section J, List of Attachments and Exhibits.

For a complete list of CDRLs, see Section F, Deliveries or Performance. In addition, CDRLs are indicated for each requirement in Part 5, Specific Tasks.

SECTION C
DESCRIPTION/SPECIFICATIONS/STATEMENT OF WORK

PART 7, ATTACHMENT 1
PERFORMANCE REQUIREMENTS SUMMARY

The Government will monitor the contractor's performance to determine whether the contractor is meeting the performance levels stated in the contract. The performance standards are specified in Part 5, C.5.1 Performance Requirements.

The Government will evaluate the contractor's performance under this contract in accordance with the Quality Assurance Surveillance Plan (QASP) (see C.5.6.2 Contract Surveillance for additional information).